Example Care Plan: HIV/AIDS & TB Co-infection

1. PLAN (Plans)

- **Current situation (Anamnesis):** Mr. Abraham (45 years old) was recently diagnosed with an HIV/AIDS infection (low CD4 count) and active pulmonary tuberculosis. He is experiencing extreme fatigue, weight loss (6 kg in one month), and has difficulty strictly adhering to his medication (combination of antiretroviral therapy (ART) and TB medication). There is a risk of infection to fellow residents/staff.
- **Care goals (SMART):**
    1. *Clinical:* Mr. Abraham achieves 100% adherence for both ART and TB medication within 2 weeks.
    2. *Safety:* No secondary TB infections occur within the institution during the infectious period.
    3. *Physical:* Mr. Abraham's weight stabilizes or increases by at least 2 kg within 4 weeks.
- **Agreed actions:**
    - *Care Team & Pharmacy:* Deployment of **Directly Observed Therapy (DOT)** ; nurses personally administer the medication and monitor intake to prevent resistance.
    - *Infection prevention:* Mr. remains in strict isolation (if infectious) in accordance with WIP guidelines. Staff wear FFP2 masks.
    - *Dietitian:* Start with energy-rich and protein-rich complementary foods (twice a day).

2. DO (Execute)

- **Implementation:** The medication is administered daily at fixed times (e.g., 08:00 and 20:00) via the DOT method. The isolation measures are strictly adhered to. Mr. receives his extra protein shakes daily .
- **Reporting (ECD):** Nurses report specifically on:
    - Taking medication and any side effects (such as nausea or skin rash).
    - Compliance with isolation and the psychological well-being of the client (in relation to social isolation).
    - Weekly weight check.

3. CHECK (Check / Evaluate)

- **Evaluation moment:** After 2 weeks (and in the interim upon lab results).
- **Findings:**
    - *Adherence to therapy:* The goal has been achieved. 100% of the medication has been successfully taken via DOT. The TB doctor reports that the bacterial load in the sputum is decreasing.

- *Safety:* No transmission to staff or fellow residents. Mr. may leave isolation as soon as the TB doctor declares the sputum negative.
- *Physical (Bottleneck):* Mr. has not gained weight and indicates that he finds the protein shakes very unpleasant. He complains of a persistent metallic taste in his mouth due to the medication.

4. ACT (Adjust / Secure)

- **Safeguarding what works well:** The DOT method for medication intake remains in effect unchanged, as adherence to therapy for HIV and TB is of vital importance to prevent resistance.
- **Adjustment (New 'Plan' phase):** The feeding plan needs to be adjusted due to the bottleneck with complementary feeding:
  - *New campaign:* The dietitian replaces the shakes with savory, protein-rich alternatives (such as specially fortified soups or quark) and oral care products are used to combat the unpleasant taste.
  - *New Check date:* Evaluation of weight in exactly 2 weeks.